

1. Administrative & Study Identification

Full Study Title: A GLOBAL PHASE 2/3 INTERVENTIONAL STUDY OF PF-08634404 IN COMBINATION WITH CHEMOTHERAPY IN PARTICIPANTS WITH EXTENSIVE STAGE SMALL CELL LUNG CANCER **Short Title/Acronym:** PF-08634404 + Chemotherapy in ES-SCLC **Protocol Number:** PF-08634404-01 (Internal Study ID) **NCT Number:** NCT07226999 **Posting ID:** 799730124609028324 **Sponsor Name:** Pfizer **Investigational Product:** PF-08634404 (bispecific anti-PD-1/VEGF antibody) **Phase of Development:** PHASE2, PHASE3 **Trial Status:** NOT_YET_RECRUITING (Not yet recruiting participants) **Medical Monitor:** Pfizer Clinical Trials Inquiry Team (PfizerClinicalTrialsInquiry@pfizer.com)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and efficacy of PF-08634404 when given with chemotherapy in adults with extensive-stage small cell lung cancer (ES-SCLC), and to compare this combination against the approved standard treatment (atezolizumab [Tecentriq] plus chemotherapy). **Secondary Objectives:** To evaluate the Duration of Response (DOR), Progression Free Survival (PFS), Overall Survival (OS) in Phase 2, and laboratory abnormalities.

2.2 Background & Rationale To address the critical unmet medical need for more effective treatments in ES-SCLC, a fast-growing lung cancer with poor prognosis. Recent studies have shown that bispecific antibodies targeting both VEGF and PD-1 can overcome immune suppression in the tumor microenvironment. This study aims to replace standard immune checkpoint inhibitors with the bispecific antibody PF-08634404 combined with chemotherapy to improve clinical outcomes and symptom management.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Atezolizumab plus chemotherapy) **Blinding:** Open-label / Double-blind (Treatment arms are compared against an active control in Part 2) **Randomization:** Randomized (2-Part Study: Part 1 for Safety Lead-in, Part 2 for Efficacy Comparison)

3.2 Planned Enrollment & Duration Target \$N\$: 550 participants **Number of Sites:** Global (United States, Japan, Puerto Rico, etc.) **Estimated Study Start:** Late 2025 **Estimated Primary Completion:** Up to approximately 2 years after the completion of study treatment for the last participant.

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically or cytologically confirmed extensive-stage small cell lung cancer (ES-SCLC). 3. Participants have not received prior systemic therapy (chemotherapy, radiotherapy, chemoradiation) for ES-SCLC. 4. Treatment-free for at least 6 months since last chemo/radiotherapy, among those treated (with curative intent) with prior chemo/radiotherapy for limited-stage SCLC. 5. Have at least one measurable lesion as the targeted lesion based on RECIST V1.1. 6. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 7. Adequate organ function.

4.2 Exclusion Criteria 1. Known active CNS lesions, including brainstem, meningeal, or spinal cord metastases or compression. 2. Leptomeningeal disease. 3. Clinically significant risk of hemorrhage or fistula. 4. History of another malignancy within 3 years. 5. Active autoimmune diseases requiring systemic treatment within the past 2 years.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Administered in repeated time periods called cycles. Some participants will continue receiving PF-08634404 as maintenance therapy alone.

Route of Administration: Intravenous (IV) infusion. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or completion of standard study timeline.

5.2 Concomitant & Prohibited Therapy Permitted: Concomitant standard-of-care chemotherapy. **Prohibited:** Other systemic anti-cancer therapies, recent prior chemotherapy for ES-SCLC, or live vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, RECIST V1.1 Imaging, Labs
Baseline	Day 0	Randomization, First IV Infusion, PK sampling
Interim	Per Cycle	Safety Labs, Efficacy Assessment, EORTC QLQ-LC13 questionnaires
End of Study	Follow-up	Final Vital Signs, Exit Interview, Long-term OS tracking (up to 2 years post-treatment)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. **Phase 2:** Confirmed Objective Response Rate (ORR) per Response Evaluation Criteria in Solid Tumors version 1.1 (RECIST 1.1) based on the investigator's assessment (defined as the proportion of participants in whom a confirmed CR or PR is observed). 2. **Phase 2:** Number of participants with

treatment-emergent adverse events (characterized by type, frequency, severity per NCI CTCAE version 5.0, timing, seriousness, and relationship). 3. **Phase 3:** Overall Survival (OS) defined as the time from the date of randomization to the date of death due to any cause.

7.2 Secondary & Exploratory Endpoints 1. Duration of Response (DOR) as assessed by Investigator based on RECIST v1.1. 2. Progression Free Survival (PFS) as assessed by investigator based on RECIST v1.1. 3. Number of participants with Laboratory abnormalities. 4. Overall Survival (OS) as a secondary outcome measure in the Phase 2 portion of the study.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Method of collection via clinical assessments, laboratory monitoring, and HCP interviews in Phase 2 and Phase 3. **Serious Adverse Events (SAEs):** Standard reporting timeline for oncological trials (typically 24 hours for life-threatening events). **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee to evaluate safety.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy endpoints (ORR, OS); Safety Set for treatment-emergent AEs and laboratory abnormalities tracking. **Sample Size Justification:** Target enrollment of 550 participants powered to demonstrate efficacy of PF-08634404 + chemotherapy vs. atezolizumab + chemotherapy. **Handling of Missing Data:** Standard handling for missing RECIST progression evaluations and PRO datasets.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: High frequency of onsite monitoring visits during the Phase 2 safety evaluation, transitioning to routine Phase 3 multi-site monitoring. **Audit Trail:** eCRF locking, tracking of infusion records, and imaging reviews via central radiologic auditing.

11. Study Identifiers & Governance

Primary ID: {{NCT07226999}} **Registry Number:** {{NCT07226999}}
Posting ID: {{799730124609028324}} **Sponsor/Lead Organization:** {{Pfizer}} **Study Title:** {{A Study to Learn About the Study Medicine Called PF-08634404 in Combination With Chemotherapy in Adult Participants With Extensive-Stage Small Cell Lung Cancer}}
Official Regulatory Title: {{A GLOBAL PHASE 2/3 INTERVENTIONAL STUDY OF PF-08634404 IN COMBINATION WITH CHEMOTHERAPY IN PARTICIPANTS WITH EXTENSIVE STAGE SMALL CELL LUNG CANCER}}

12. Clinical Framework (ES-SCLC Specific)

Primary Condition: {{Small Cell Lung Cancer (SCLC) / Extensive-Stage Small Cell Lung Cancer (ES-SCLC)}} **Preferred Terminology:** {{Small cell carcinoma of lung}} **Diagnosis Threshold:** {{Histological or cytological confirmation of ES-SCLC}} **Medical Methodology:** {{Anti-PD-1 x VEGF Bispecific Antibody combination therapy vs. Atezolizumab (Tecentriq) combination}} **Optimization Target:** {{Objective Response Rate and Overall Survival improvement}}

13. Study Procedures & Methodology

Management Pathway: {{Standard clinical pathway for first-line ES-SCLC}} **Education Protocol (HCP):** {{Phase 2 safety profiling and RECIST V1.1 tumor assessment training}} **Education Protocol (Patient):** {{Onsite training for reporting adverse events and symptoms}} **Quality Audit Mechanism:** {{Central imaging review and blinded independent central review (BICR)}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to ~2 years after completion of study treatment}} **Onsite Visit Cadence:** {{Aligned with IV infusion cycles}} **Remote Monitoring Frequency:** {{Continuous survival follow-up}} **Checklist Review Interval:** {{Prior to each infusion cycle}}

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				